



PERSONA

Director Dale

Economic Buyer · EMS-primary / Nursing-secondary

VRpatients Brand Foundation · Audit date 2026-05-22 · Pre-questionnaire base draft · Run 2 of 6

Confidence tags **[BEDROCK]** evidence-locked **[CRM-BACKED]** CRM-supported **[ASSUMPTION]** needs human input

1. Identity

Director Dale is an EMS-program director (or sim-lab director) at a community college or municipal training agency who signs the PO when a state grant, accreditation deadline, or aging manikin forces a decision — and who has to defend that spend to a board, an accreditor, or a fire chief who never saw the demo. **[CRM-BACKED]**

Handle changed from the web-research file's implied Dean/Director. CRM contact-title counts: Coordinator ~427, Director ~414, Chief ~92, Dean ~47 (noisy due to first-name overlap). Director outnumbers Dean ~9-to-1 across 18,040 populated job titles.

2. Role Type

Buyer (primary) + Influencer (secondary). Signs procurement. Touches the simulation experience occasionally; touches the authoring tool rarely. Acts on the recommendation of a Sim Lab / Training Coordinator who ran the demo. CRM cannot validate the buyer-vs-influencer split directly: hs_buying_role is populated on 0 of 23,014 contacts. **[ASSUMPTION]**

3. Snapshot

- **Segment.** EMS-primary (CRM 82% of closed-won, N=337). Nursing-side institutional buyer exists at ~12% — but wins at roughly half the rate of EMS (Nursing ~17% close rate vs EMS ~40% on closed deals). **[CRM-BACKED]**
- **Institutional setting.** Bimodal. Community College = 43% of populated sub-industries; Hospital = 36% (N=241). Not the single-track community-college audience the web research implies. **[CRM-BACKED]**

- **Tech comfort.** Low-to-moderate. Risk-averse. Likely been burned by past edtech that died after the champion left or the next budget cycle hit. **[ASSUMPTION]**
- **Typical check size.** \$5K–\$30K is the dense band. Two outlier Nursing closed-wons reached \$152K and \$330K. Sales-cycle median for wins is 2–6 months — much shorter than the web-research 9–18 month estimate. **[CRM-BACKED]**

4. Goals

- **Functional.** Hit accreditation thresholds (CoAEMSP 70% on NREMT outcomes for EMS; NCLEX first-time pass rate for Nursing) and produce defensible outcome reporting at the next site visit. **[ASSUMPTION]**
- **Emotional.** Be the director on whose watch the program got modern — not the one on whose watch it got put on probation or lost accreditation. **[ASSUMPTION]**
- **Social.** Be named in state workforce announcements; be seen as the forward-leaning leader by peers at NAEMSE, IMSH, and state EMS associations. **[ASSUMPTION]**

5. Pains (ranked, most acute first)

1. **“Limited Budget” — but with hidden subtext.** 70 of 309 populated loss reasons (23%) tag this — the #1 substantive loss reason after the 42% “No Response” ghost tag. Almost certainly masks champion-couldn’t-sell-internally, grant-cycle-missed, or deeper objections that reps don’t tag. The CRM tells us the WHAT; the WHY is open. **[CRM-BACKED]**
[ASSUMPTION]
2. **The “expired and didn’t renew” tail.** Roughly 150 historical New Business wins now sit in expired or non-renewed status. The buyer who signed once doesn’t sign twice unless the product actually got used. Post-sale stickiness — not first-purchase persuasion — is the product story. **[CRM-BACKED]**
3. **Defending the purchase to people who didn’t see the demo.** Boards, accreditors, peer chiefs. They need a one-sentence justification that doesn’t depend on having put on a headset. Marketing collateral has to do that work. **[ASSUMPTION]**
4. **Content-gap objections from advisory boards.** Pediatrics being the canonical example. Only 8 of 309 populated loss reasons (3%) tag “Missing features” — but 5 of those 8 are Nursing-side, overrepresenting nursing in feature-gap losses by roughly 2× relative to nursing’s share of closed-lost. **[CRM-BACKED]** **[ASSUMPTION]**
5. **Procurement and approval-chain friction.** Most felt above the ~\$10K threshold where additional sign-offs kick in (see Tech-Hat Tasha Q15). The economic buyer absorbs this pain even when the technical buyer never sees it. **[ASSUMPTION]**

6. What They’ve Tried Before & Why It Failed

- **High-fidelity manikins (SimMan family, SimBaby, SimJunior).** Throughput-limited (1:1 student to manikin), \$27K–\$60K+ acquisition, multi-thousand-dollar annual service contracts, faculty time required to author scenarios. **[CRM-BACKED]**
- **Past edtech that died on the shelf.** Bought once, deployed once, dropped after the champion left or the budget cycle came up. This is the pattern showing up inside VRpatients’ own ~150 expired customers. **[CRM-BACKED]**
- **Free or low-cost screen-based sim.** Insufficient fidelity to satisfy accreditor expectations; can’t be defended on a site visit. **[ASSUMPTION]**

Gap VRpatients fills: a per-learner subscription that doesn't require an additional FTE to operate, breaks the 1:1 throughput ceiling, and produces accreditation-defensible outcome reporting. **[ASSUMPTION]**

7. Trigger Events

- **NREMT pass rate drops below the 70% CoAEMSP threshold** (EMS) or NCLEX first-time pass rate drops below state threshold (Nursing). Accreditation alarm — the strongest narrative trigger in the web research; not yet visible in CRM free-text. **[ASSUMPTION]**
- **Existing high-fidelity manikin fails or goes off-warranty.** A \$30K+ repair-or-replace decision lands on the director's desk. **[ASSUMPTION]**
- **Perkins V / HRSA NEPQR-SET / state workforce grant RFP opens.** Typically annual, spring submission, summer/fall award. **[ASSUMPTION]**
- **Hospital clinical-placement slots reduced or rescinded.** Forces program to find an alternative for clinical-hour requirements. **[ASSUMPTION]**
- **Cohort expansion mandated without a corresponding faculty FTE.** Director must find throughput somewhere. **[ASSUMPTION]**
- **A peer program announces a VR lab in local press.** Competitive pressure from the next county over. **[ASSUMPTION]**

8. Objections (top 3, categorized)

1. **"You don't have pediatrics."** — Won't work for me. Small slice of recorded losses (5 of 8 missing-feature losses = nursing-skewed) but surfaces consistently in pre-buy conversations. The authoring tool is not the answer — buyers read "build it yourself" as a labor tax. MVP in testing per Tasha Q1. **[CRM-BACKED]** **[ASSUMPTION]**
2. **"Limited budget — capital cycle closed in May"** or **"we just bought SimMan."** — Wrong timing / Won't work for me. The #2 named loss reason after "No Response." Masks champion-couldn't-sell-internally on an unknown but likely meaningful fraction. **[CRM-BACKED]**
3. **"Who is Virtual Education Systems? What happens to my scenarios if you go out of business?"** — Don't trust the vendor. Especially live in larger institutional buyers; the Nightingale-scale outlier suggests it's surmountable but requires substantive trust-building. **[ASSUMPTION]**

9. Watering Holes (named)

- **EMS accreditation triad.** CoAEMSP, NREMT, CAAHEP. **[ASSUMPTION]**
- **EMS conferences.** NAEMSE Symposium (Aug), EMS World Expo, FDIC, IMSH (Jan, ~4,300 attendees — dean-rare, coordinator-rich). **[ASSUMPTION]**
- **Nursing side (secondary).** INACSL Annual (June), NLN Education Summit (Sep), ACEN/CCNE. **[ASSUMPTION]**
- **Trade press.** HealthySimulation.com (Lance Bailly — controls the trade narrative deans actually read). **[ASSUMPTION]**
- **State EMS associations.** State-by-state — high-leverage and underused channel for a regional-buyer persona. **[ASSUMPTION]**

- **Names whose endorsement moves a buyer.** George Hatch (CoAEMSP Executive Director), Bill Seifarth (NREMT Executive Director); on the nursing side, Suzie Kardong-Edgren and KT Waxman. **[ASSUMPTION]**

10. Voice of Customer

Pull quotes are forecasted from web research and inferred from the deal patterns; not yet verified verbatim in VRpatients CRM free-text.

“We need to defend this on the next site visit.”

Forecasted from accreditation-driven buying pattern · **[ASSUMPTION]**

“Our pass rate dropped last cohort.”

Forecasted trigger phrase · **[ASSUMPTION]**

“What's the per-learner cost?”

Forecasted pricing-model question · **[ASSUMPTION]**

“Can the grant cover this?”

Forecasted grant-funding inquiry · **[ASSUMPTION]**

“What does it take to keep using it after the first year?”

Forecasted renewal-anxiety question · **[ASSUMPTION]**

Plausible 11pm Google search: “Perkins V eligible simulation equipment 2026” or “CoAEMSP simulation requirements paramedic.” **[ASSUMPTION]**

11. Before / After

BEFORE VRpatients works

- Defends a 1:1-student manikin program with a five-figure annual service contract, faculty hours bottlenecked on authoring, and an advisory board asking “what about pediatrics?” **[CRM-BACKED]**
- Renewal conversation is “do we want to spend on this again?” — and the answer is increasingly “no.”

AFTER VRpatients works

- Defends a per-learner subscription where every student in the cohort gets the same standardized encounter and scenarios run unattended. **[CRM-BACKED]**
- Renewal conversation becomes “this is the line item that fixed our pass rate.” The advisory board still asks “what about pediatrics?” — but the answer is now “we have it” (assumes peds MVP ships per Tasha Q1). **[ASSUMPTION]**

After VRpatients fails: headsets sit on a shelf, platform isn't used, month-10 renewal goes silent. This is what the ~150 expired deals look like in the CRM today. The product story has to bake in stickiness — not just first-sale persuasion. **[CRM-BACKED]**

12. Confidence Ledger

[BEDROCK]

- VRpatients sells primarily into healthcare-education programs.
- Deals close in the \$5K–\$30K band with rare six-figure outliers.
- Community college is a meaningful institutional setting.

[CRM-BACKED]

- EMS dominates closed-won at 82% (N=337).
- Nursing wins at ~17% vs EMS ~40% on closed deals.
- “Limited Budget” is the #1 substantive loss reason (70 of 309 populated, 45.6% population rate).
- ~150 historical wins now sit in expired or non-renewed status.
- Sales-cycle median for wins is 2–6 months, not 9–18.
- Contact-title counts: Director ~414, Coordinator ~427, Chief ~92, Dean ~47.

[ASSUMPTION]

- Every WHY behind a CRM WHAT — what “Limited Budget” actually masks.
- Exact signer title at EMS deals (Director vs Chief vs Coordinator).
- Root cause of Nursing's 2× loss rate.
- Whether Skeptic is a separate persona or a stance.
- Watering-holes / influencer / VOC lists match how customers actually found us.
- NCLEX / NREMT / accreditation as primary triggers (web-asserted, unverified in CRM).

Open Questions Routed to Team

Full traceable list in brand/personas/persona-audit-open-questions.md → “Director Dale” section. Summary by team:

- **Sales.** Anchor title for EMS persona — Director, Chief, or Coordinator? (D1); What “Limited Budget” really means when reps tag it (D5).
- **Sales + Product + CS.** Root cause of Nursing 2× loss rate (D2).
- **Sales + CS + Leadership.** Buying motion on the two outlier Nursing mega-deals (D3).
- **Sales + Leadership.** Hospital sub-industry — adjacent segment or noise? (D4).
- **Leadership + Sales + Marketing.** Should Economic Buyer split into EMS / Nursing personas? (D6).
- **Leadership + Marketing.** Skeptic — separate persona or stance? (D7).
- **CS + Sales.** At renewal, is the original buyer still the right re-engagement contact? (D8).

Composite persona. No single named customer was used. Aggregate patterns only. Pre-questionnaire base draft — not for promotion to outputs/final-deliverables/ until the sales / CS / product / leadership questionnaire closes the [ASSUMPTION] gaps.